

FRAUD ANALYTICS & INTELLIGENCE REPORT

ECHS FRAUD ANALYTICS

Module 13 — High-Value Claim Risk Scoring

CLAIMS ABOVE ₹5 LAKH | LAST 5 YEARS | RULE-BASED SIGNALS +
COMPOSITE RISK SCORE

HIGH-VALUE CLAIMS	TOTAL EXPOSURE	DEDUCTED	100% DEDUCTED	FLAGGED LOGINS
74,593	₹5,630.76 Cr	₹1,785.08 Cr	9,717	392

IIT KANPUR — Data Analytics & Fraud Intelligence Division

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Executive Summary

Module 13 targets the highest-financial-risk stratum of the ECHS claim ecosystem: individual claims exceeding **₹5 lakh** per admission, analysed across the **last five years** from the **claim_intimation** and **claim_submission** tables. Each claim, hospital login and beneficiary card is scored with a transparent, rule-based composite risk score (0–100) — no machine learning, so every score is auditable. **Total Financial Exposure** is the gross claimed value of the flagged claims (approximate; the recoverable amount is set at audit).

TOTAL EXPOSURE ₹5,630.76 Cr gross claimed, >₹5L	TOTAL DEDUCTED ₹1,785.08 Cr 31.7% overall	100% DEDUCTED 9,717 approved at ₹0	CHRONIC CLAIMANTS 3,135 ≥3 high-value claims	BULK EVENTS 19 5 system-compromise
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By composite risk band, **132** beneficiary cards and **15** hospital logins fall in the CRITICAL/HIGH tiers and head the verification queue. Flagged claims span 2011-09-08 to 2026-03-31.

Fraud patterns at a glance

#	Pattern	What it detects	Count	Risk
1	High-value claims (>₹5L)	Exposure concentration in the top claim stratum	74,593	CRITICAL
2	100% deduction	Multi-lakh bills approved at ₹0 — billing beyond the package ceiling	9,717	CRITICAL
3	Hospital overcharging	Logins with avg deduction > 25% on high-value claims	392 logins	CRITICAL
4	Chronic repeat claimants	One card with ≥3 high-value claims	3,135	HIGH
5	Same-day duplicate billing	Same card+date+amount under multiple intimation IDs	71 groups	CRITICAL
6	Bulk claim injection	Hundreds of intimation IDs created in one day (programmatic)	19 events	CRITICAL
7	Ghost / anomalous hospital ID	High-value claims with NULL or phone-like hospital IDs	23,959	HIGH
8	ECHS polyclinic abuse	Polyclinic logins routing high-value IPD claims (high deduction)	4,499	HIGH

Immediate recommended actions

1. **Treat bulk-injection events as a security incident** (19 events) — pull portal access logs and escalate to ECHS IT / CERT-In.
2. **Recover on same-day duplicates** (71 groups) — confirm which submissions were paid and reclaim the surplus.
3. **Physically verify the top chronic claimants** (3,135 cards ≥3 claims) — discharge-summary sampling.
4. **Audit the top-deduction hospital logins** (392 logins >25% deduction) — empanelment + package-rate review.
5. **Reject invalid hospital IDs at intake** (23,959 claims, ₹1,729.34 Cr) — block NULL/phone-like IDs.

HOW THE ANALYSIS WORKS

Parameters & Methodology

Data. High-value claims (gross claimed > ₹5 lakh) over the last five years, joined from **claim_intimation** and **claim_submission** on the intimation ID. Regional figures use the pre-aggregated **settlement_stat** table.

Claim pipeline. A claim flows: intimation (admission notified) → submission (hospital bills the gross amount) → UTI audit (an approved amount is set; the balance is the **deduction**) → settlement. A 100% deduction means the audit approved nothing on a multi-lakh bill.

Table P.1 — Parameters & definitions

Parameter	Definition / threshold
High-value claim	Gross claimed amount > ₹5,00,000 for a single admission
Deduction %	$(\text{gross claimed} - \text{UTI approved}) / \text{gross claimed} \times 100$
Chronic repeat claimant	One beneficiary card with ≥ 3 high-value claims
Single-hospital lock-in	Share of a card's high-value claims filed at one hospital login
Bulk injection (tiers)	Same-day intimations per card: >300 = system compromise; 100–300 = account abuse; 50–100 = watch
Anomalous hospital ID	NULL, -1/0 placeholder, or a numeric/phone-like value (not a real login)
Total Financial Exposure	Sum of gross claimed on flagged claims (approximate; recovery set at audit)

Composite risk score — component weights (0–100, no ML)

Bands: **CRITICAL** ≥ 70, **HIGH** 45–69, **MEDIUM** < 45. Each score is a plain weighted sum of normalised signals — fully reconstructable from the source rows.

Claim-level score

Component	Weight
Deduction	35
Magnitude	25
Full Deduction	15
Same Day Duplicate	15
Anomalous Hospital	10

Hospital-level score

Component	Weight
Avg Deduction	40
Absolute Deducted	30
Volume	15
Full Deduction Share	15

Claimant-level score

Component	Weight
Volume	35
Single Hospital Lockin	25
Total Claimed	25
Bulk Injection	15

Limitations & Assumptions

- **Red flags, not verdicts.** Every item is an automated investigative lead, not a confirmed finding. A qualified auditor must verify each case before any action.
- **Deduction is not fraud.** A high deduction can reflect a legitimate package ceiling, a coding error, or incomplete documentation — it flags where billed value was removed, not why.
- **Login codes are identifiers.** Hospital login codes (e.g. parkhosg, pol.3325) are used as identifiers only; this report does not assert their real-world hospital identity.
- **Data quality.** Findings depend on the accuracy of the intimation/submission tables and the UTI deduction codes; systemic gaps (e.g. missing hospital IDs) can both create and hide signals.
- **Scope.** Analysis covers the last five years only; older claims are out of scope by design.

ALL CASES REQUIRE VERIFICATION BY QUALIFIED AUDITORS BEFORE ANY ACTION IS TAKEN.

Q13 · DATA-DERIVED

Key Fraud Signals

Derived directly from the last-5-year scan;
risk band reflects the composite score

#	Signal	Finding	Risk
1	Largest single high-value claim	₹99.48 L at hos.3651 (HEM JOSHI); 100.0% deducted.	CRITICAL
2	Highest hospital deduction rate	pol.3325: 74.34% avg deduction on 74 high-value claims.	CRITICAL
3	Largest absolute hospital deduction	parkhosg: ₹64.43 Cr deducted across 2,399 claims.	HIGH
4	100% deduction claims	9,717 high-value claims approved at ₹0 — billing beyond the package ceiling.	HIGH
5	Same-day duplicate billing	BHISHAM DEO SHUKLA (card CH000003881057): 2 identical ₹32.73 L claims on 2022-11-11.	CRITICAL
6	Chronic repeat claimant	BHUPAL SINGH (card 000001724273): 117 HV claims, ₹7.13 Cr exposure, 86% at fortis@D1.	CRITICAL
7	Extreme bulk claim injection	GURJIT SINGH: 546 intimation IDs in one day at p.fatesahi (SYSTEM COMPROMISE).	CRITICAL
8	Anomalous / missing hospital IDs	23,959 high-value claims (₹1,729.34 Cr) carry NULL or phone-like hospital IDs — probable ghost admissions.	HIGH

REPRESENTATIVE FLAGGED CASES

Case Studies

Drawn directly from the data; each is a lead for verification, not a verdict

Case 1 — Chronic Repeat Claimant

Beneficiary	BHUPAL SINGH (card 000001724273)
Pattern	117 high-value claims; 86% at one login (fortis@D1)
Exposure	₹7.13 Cr claimed / ₹6.73 Cr approved (5.5% deducted)
Window	2021-10-05 to 2026-03-20 (~54 months; on average one admission every 14 days)

BHUPAL SINGH filed **117 separate high-value claims** in about 54 months — on average one admission every **14 days**. The low 5.5% deduction means most claims pass audit, so the test is documentary: either a genuine chronic condition needing case management, or manufactured admission episodes generating a fresh base fee each time.

Action: pull discharge summaries for a 20-claim sample and reconcile admission dates / ICU notes (14 days).

Case 2 — Ghost-Hospital Repeat Claimant

Beneficiary	AJAYA KUMAR NAYAK (card 000005455660)
Pattern	45 high-value claims; primary hospital ID = NULL (untraceable)
Exposure	₹5.96 Cr claimed (28.9% deducted)

Every one of AJAYA KUMAR NAYAK's 45 high-value claims (₹5.96 Cr) carries no traceable hospital ID — a classic **ghost-admission** pattern; the partial deduction shows the system flags but does not fully reject them.

Action: trace the servicing facility from settlement/bank records; if none exists, treat as fabricated (30 days).

Case 3 — Same-Day Duplicate Billing

Beneficiary	BHISHAM DEO SHUKLA (card CH000003881057)
Duplicate	2 identical ₹32.73 L claims on 2022-11-11 at 8983336860
Intimation IDs	23367707, 23367744

The same card, admission date and amount (₹32.73 L) were submitted under 2 different intimation IDs — the textbook signature of duplicate billing to collect reimbursement more than once.

Action: confirm whether more than one submission was paid, recover the surplus, sweep the login for other duplicates (7 days).

Case 4 — Possible Card-ID Manipulation

Beneficiary	AMRIT LAL CHADHA
Two cards	DL1000004517957 vs DL2000004517957 (differ by one character)
Combined	7 high-value claims, ₹2.00 Cr, at p.delhicnt

The same beneficiary name appears on two card numbers that differ by a single character (**DL1000004517957** vs **DL2000004517957**). This can indicate duplicate card issuance / card-ID manipulation used to split a fraud trail — or, less likely, two distinct people sharing a common name. It is a lead, not a conclusion.

Action: verify both cards against the service record — one pensioner or two? (immediate).

Case 5 — Extreme Bulk Claim Injection

Beneficiary	GURJIT SINGH
Event	546 intimation IDs in ONE day (2022-08-04) at p.fatesahi
ID range	21939246 – 21939907 (near-consecutive)
Tier	SYSTEM COMPROMISE

546 claim-intimation records created for one card on a single day is far beyond manual web-portal entry. The near-consecutive ID range is the signature of programmatic batch insertion — portal credential compromise or insider database access, not ordinary claim fraud.

Action: obtain portal access logs (IP/session/timestamps) for that date, suspend the login, escalate to ECHS IT / CERT-In (immediate).

Q13A · CLAIM-LEVEL

Top High-Value Individual Claims

Ranked by gross claimed (exposure). Ded%
= (claimed – approved) / claimed

Table Q13a.1 — Top 30 high-value claims

Risk	Beneficiary	Hospital	Admission	Diagnosis	Exposure	Ded%
CRITICAL 74.9	HEM JOSHI	hos.3651	2024-08-22	chest cengestion and fever si...	₹99.48 L	100.0
CRITICAL 81.7	SURENDRA SINGH	6206305629	2023-04-06	medical bill calaim	₹86.88 L	100.0
HIGH 68.5	HARISH KUMAR SHARMA	pol.1928	2023-03-17	RIGHT SIDED INTRACEREBRAL BLE...	₹74.14 L	100.0
HIGH 68.3	ROHTASH KUMAR	metro	2023-07-05	SUDDEN LOC	₹73.32 L	100.0
HIGH 66.8	AMRIT LAL CHADHA	p.delhicnt	2023-04-27	LRTI , COPD ,OSA ,DM ,AF	₹67.16 L	100.0
HIGH 66.6	RAMESH NAGPAL	p.lodhird1	2025-03-22	ACUTE ISCHEMIC STROKE	₹66.53 L	100.0
CRITICAL 76.4	PAWAN KUMAR	NULL	2025-10-18	CASE OF DIFFUSE LARGE B-CELL ...	₹65.48 L	100.0
MEDIUM 37.2	NARESH KUMAR BUDHWAR	p.rohtak	2023-06-29	Interstitial lung disease wit...	₹63.67 L	60.73
HIGH 65.9	ARUN KUMAR SHOREY	p.gurgaon	2023-11-12	LRTI WITH RESPIRATORY FAILURE...	₹63.64 L	100.0
CRITICAL 75.6	NIRANJAN DAS	8327741223	2025-07-18	MEDICINE	₹62.24 L	100.0
CRITICAL 74.5	APURBA MAITY	9757020120	2025-12-03	Treatment for provisional dia...	₹58.11 L	100.0
CRITICAL 74.5	OM SINGH	NULL	2025-01-06	EARLY RELAPSED DLBCL	₹58.10 L	100.0
MEDIUM 40.9	RAJARAM RAJA SANJAY	p.chennai	2024-09-10	ACUTE PULMONARY EDEMAACS A...	₹56.21 L	76.77
HIGH 64.0	SURESH CHANDRA UPADHYAY	hos.4364	2024-07-17	Lower Respiratory Tract Infec...	₹55.86 L	100.0
HIGH 63.9	M S GANGA RAO	p.adolfp	2023-04-07		₹55.68 L	100.0
HIGH 63.8	PREM SHANKAR TEWARI	pol.3341	2023-09-24	SEPTIC SHOCK, DECOMPENSATED C...	₹55.30 L	100.0
HIGH 63.7	OMKAR SINGH	metro	2023-05-29	sudden onset of severe headac...	₹54.84 L	100.0
HIGH 63.4	GYAN BAHADUR CHHETRI	hos.5750	2025-05-15	SUSPECTED HYPOXIC BRAIN INJU...	₹53.79 L	100.0

Risk	Beneficiary	Hospital	Admission	Diagnosis	Exposure	Ded%
HIGH 63.4	SURYA KANT	metro	2025-10-13	SEVER COUGH AND COLD,WEIGHT L...	₹53.74 L	100.0
MEDIUM 32.1	M RAMESH	p.secundm	2022-01-09		₹53.61 L	53.37
HIGH 63.2	BRAJ NARAYAN SINGH	medantae	2023-03-18	Fever under evaluation.	₹52.63 L	100.0
HIGH 63.1	PARMOD	metro	2024-05-28	high grade fever	₹52.54 L	100.0
HIGH 63.1	SITA RAM VERMA	vhasha16	2022-10-14	AS PER ER PERSCRIPTION	₹52.21 L	100.0
HIGH 62.9	YOGESH KUMAR	hos.4245	2025-11-01	MEDICAL MANAGEMENT	₹51.41 L	100.0
HIGH 62.6	VEER SINGH GANGWAR	medantae	2023-10-01	ARDS/SEPSIS/SEPTIC SHOCK.	₹50.48 L	100.0
HIGH 62.4	PAWAN KUMAR	yhgnech2	2025-01-14	C/O SUDDEN ONSET RIGHT SIDE W...	₹49.56 L	100.0
HIGH 62.1	RAM SEWAK	p.agra1	2025-06-09		₹48.58 L	100.0
HIGH 62.1	SHIV DUTT	shrcechs	2023-07-20	CLL	₹48.38 L	100.0
MEDIUM 18.3	ATUL SEHGAL	p.secundm	2023-01-07		₹48.27 L	17.91
CRITICAL 72.0	VIJAI BAHADUR YADAVA	NULL	2025-06-07	FOR ALLOGENIC HAPLO-IDENTICAL...	₹48.11 L	100.0

Ghost & anomalous-hospital claims

High-value claims whose hospital ID is missing (NULL), a placeholder, or a phone number — i.e. no traceable provider. These are prime ghost-admission leads.

Table Q13a.2 — Top ghost / anomalous-hospital claims

Beneficiary	Hospital ID	Type	Admission	Exposure	Ded%
SURENDRA SINGH	6206305629	Phone-like	2023-04-06	₹86.88 L	100.0
PAWAN KUMAR	NULL	NULL (missing)	2025-10-18	₹65.48 L	100.0
NIRANJAN DAS	8327741223	Phone-like	2025-07-18	₹62.24 L	100.0
APURBA MAITY	9757020120	Phone-like	2025-12-03	₹58.11 L	100.0
OM SINGH	NULL	NULL (missing)	2025-01-06	₹58.10 L	100.0
VIJAI BAHADUR YADAVA	NULL	NULL (missing)	2025-06-07	₹48.11 L	100.0
SUMER SINGH	NULL	NULL (missing)	2024-02-08	₹45.87 L	100.0
SHAIK RIYAJ BASHA	NULL	NULL (missing)	2025-08-21	₹45.16 L	100.0
GORE DEVIDAS BHIKAJI	NULL	NULL (missing)	2023-05-18	₹44.92 L	100.0
TIBURCIO MENEZES	7039463672	Phone-like	2023-12-06	₹43.36 L	100.0
GAJENDER SINGH TOMAR	NULL	NULL (missing)	2025-02-28	₹41.35 L	100.0

Beneficiary	Hospital ID	Type	Admission	Exposure	Ded%
HEM CHANDRA	NULL	NULL (missing)	2024-04-27	₹40.92 L	100.0

Q13B · PROVIDER-LEVEL

Logins with ≥10 high-value claims and avg deduction > 25%, ranked by absolute deduction

Hospital Risk Scorecard — Top 20

Table Q13b.1 — Top 20 hospital logins by absolute deduction

Rank	Login	HV Claims	Exposure	Deducted	Avg Ded%	100% Ded	Risk
1	parkhosg	2,399	₹186.91 Cr	₹64.43 Cr	34.47	224	HIGH 60.2
2	metro	892	₹80.90 Cr	₹42.12 Cr	52.07	242	HIGH 57.6
3	metrc2	1,317	₹103.19 Cr	₹35.41 Cr	34.32	116	HIGH 45.4
4	medantae	1,017	₹94.52 Cr	₹33.32 Cr	35.25	136	HIGH 45.0
5	met13pvg	1,449	₹114.24 Cr	₹33.24 Cr	29.1	121	MEDIUM 42.4
6	yhgnech2	612	₹53.15 Cr	₹22.98 Cr	43.23	79	MEDIUM 42.3
7	ivyhosp	867	₹72.82 Cr	₹22.13 Cr	30.38	103	MEDIUM 37.3
8	MHCIPV	558	₹49.55 Cr	₹21.17 Cr	42.72	106	MEDIUM 42.0
9	vhasha16	536	₹52.71 Cr	₹20.04 Cr	38.02	98	MEDIUM 39.4
10	hos.4245	341	₹29.88 Cr	₹15.41 Cr	51.57	73	MEDIUM 42.3
11	jaypeerc2	388	₹37.15 Cr	₹15.28 Cr	41.13	85	MEDIUM 38.3
12	healing	875	₹55.93 Cr	₹14.87 Cr	26.59	50	MEDIUM 31.5
13	parashos	327	₹29.92 Cr	₹14.52 Cr	48.51	89	MEDIUM 41.4
14	hos.2256	421	₹36.61 Cr	₹14.09 Cr	38.5	82	MEDIUM 36.5
15	sksoni	589	₹43.87 Cr	₹13.70 Cr	31.24	95	MEDIUM 33.6
16	hos.2179	740	₹50.31 Cr	₹13.66 Cr	27.15	42	MEDIUM 30.8
17	artemis1	521	₹43.77 Cr	₹13.47 Cr	30.78	75	MEDIUM 32.8
18	emc51296	479	₹35.92 Cr	₹12.19 Cr	33.93	76	MEDIUM 33.5
19	ivyasr@jal	448	₹37.63 Cr	₹10.69 Cr	28.41	40	MEDIUM 29.4
20	hos.4364	165	₹17.59 Cr	₹10.68 Cr	60.72	62	MEDIUM 44.8

Hospital deep-dives

parkhosg — 2,399 high-value claims, ₹186.91 Cr exposure, ₹64.43 Cr deducted (34.47% average), of which **224** were approved at ₹0. Beneficiaries with a 100% deduction here include SPS PANWAR (₹28.66 L), BHARAT SINGH (₹25.00 L), SATVIR SINGH (₹24.41 L), VIJAY KUMAR (₹22.29 L), BEH RAM SINGH (₹21.95 L), SOHAN LAL (₹19.20 L). Different beneficiaries at one login all hitting 100% deduction points to systematic billing above the package ceiling rather than isolated patient fraud.

metro — 892 high-value claims, ₹80.90 Cr exposure, ₹42.12 Cr deducted (52.07% average), of which **242** were approved at ₹0. Beneficiaries with a 100% deduction here include ROHTASH KUMAR (₹73.32 L), OMKAR SINGH (₹54.84 L), SURYA KANT

(₹53.74 L), PARMOD (₹52.54 L), TEJ RAM (₹47.32 L), NARANDER NATH SHARMA (₹45.98 L). Different beneficiaries at one login all hitting 100% deduction points to systematic billing above the package ceiling rather than isolated patient fraud.

metrc2 — 1,317 high-value claims, ₹103.19 Cr exposure, ₹35.41 Cr deducted (34.32% average), of which **116** were approved at ₹0. Beneficiaries with a 100% deduction here include DARWAN SINGH (₹33.79 L), ATTAR SINGH (₹31.44 L), CHANDER DHAR (₹28.22 L), BRAHMA CHANDRA (₹25.71 L), RAM ISHWAR BHAGAT (₹24.27 L), RAGHUVIR SINGH (₹23.92 L). Different beneficiaries at one login all hitting 100% deduction points to systematic billing above the package ceiling rather than isolated patient fraud.

ECHS polyclinic login pattern

Login codes of the form **p.** / **pol.** (ECHS polyclinics) carry 4,499 high-value claims with ₹237.10 Cr deducted at a 57.53% average — far above private-hospital norms. Highest-deduction polyclinic logins: p.giddalur (81.56%), pol.3341 (80.15%), p.kumba (76.96%), p.agra1 (75.19%), p.golconda (75.05%). Polyclinic credentials routing high-value IPD claims that are then almost entirely rejected suggests these internal channels are being used to bypass pre-authorisation controls.

Q13C · REGION-LEVEL

Regional Risk Distribution

From the pre-aggregated settlement statistics; a region-level proxy

Table Q13c.1 — Regional claimed value and deduction rate (top 16 by exposure)

Region	ECHS Command	Claims	Exposure	Deducted	Ded%
1	NEW DELHI	2,477,067	₹5,259.84 Cr	₹482.07 Cr	9.17
9	PUNE	957,803	₹4,242.99 Cr	₹280.18 Cr	6.6
4	BANGALORE	1,398,702	₹3,639.80 Cr	₹209.50 Cr	5.76
8	JAIPUR	1,388,115	₹3,295.25 Cr	₹554.97 Cr	16.84
29	Region 29	1,919,597	₹3,033.42 Cr	₹348.87 Cr	11.5
27	Region 27	1,053,792	₹2,730.28 Cr	₹305.53 Cr	11.19
24	Region 24	659,799	₹2,015.48 Cr	₹192.28 Cr	9.54
30	Region 30	918,795	₹1,376.91 Cr	₹125.07 Cr	9.08
7	DEHRADUN	1,436,946	₹1,285.61 Cr	₹87.53 Cr	6.81
2	MUMBAI	1,944,603	₹1,199.17 Cr	₹87.59 Cr	7.3
3	KOLKATA	670,906	₹1,028.64 Cr	₹143.27 Cr	13.93
17	Region 17	646,784	₹859.70 Cr	₹91.98 Cr	10.7
6	CHENNAI	271,050	₹826.39 Cr	₹155.41 Cr	18.81
5	HYDERABAD	526,668	₹713.08 Cr	₹87.80 Cr	12.31
12	PATNA	350,304	₹685.19 Cr	₹38.14 Cr	5.57
15	Region 15	466,128	₹680.27 Cr	₹54.79 Cr	8.05

Regional implications

CHENNAI shows the highest deduction rate (18.81%) — the command where claim scrutiny removes the largest share of billed value. **NEW DELHI** carries the largest absolute exposure (₹5,259.84 Cr): a concentration of high-volume hospital logins makes it the biggest absolute leakage surface even at a moderate rate. High-deduction commands warrant a region-level audit of their top high-value hospitals.

Q13D · BENEFICIARY-LEVEL

Chronic Repeat High-Value Claimants

Cards with ≥3 high-value claims, ranked by total exposure. Lock-in = share at a single login

Table Q13d.1 — Top 20 chronic repeat claimants

Risk	Beneficiary	Card	Claims	Exposure	Deducted	Primary Hosp	Lock-in
CRITICAL 81.6	BHUPAL SINGH	000001724273	117	₹7.13 Cr	₹0.39 Cr	fortis@D1	86%
CRITICAL 74.0	AJAYA KUMAR NAYAK	000005455660	45	₹5.96 Cr	₹1.72 Cr	NULL	100%
HIGH 65.5	PIARE LAL	000004046297	49	₹5.55 Cr	₹0.38 Cr	NULL	69%
HIGH 63.6	JOGINDER SINGH	000003916849	72	₹4.21 Cr	₹0.09 Cr	medantae	69%
HIGH 63.4	MALKIT SINGH	000006128034	59	₹3.48 Cr	₹0.00 Cr	FHLDH123	85%
HIGH 57.6	PUSHKAR SINGH JETHI	000004735826	53	₹3.10 Cr	₹0.02 Cr	medantae	70%
HIGH 60.5	KASHMIR SINGH	000003083761	52	₹3.06 Cr	₹0.00 Cr	echsfeha	83%
HIGH 58.3	KHIM SINGH	000002488287	53	₹3.04 Cr	₹0.07 Cr	msshshb1	74%
HIGH 61.0	PARAS RAM	000003100630	52	₹2.93 Cr	₹0.08 Cr	NULL	86%
HIGH 52.4	RAJ SINGH	000003881511	43	₹2.56 Cr	₹0.02 Cr	SMVDNSH	63%
HIGH 48.8	SATYAVIR SINGH	000002571829	38	₹2.49 Cr	₹0.30 Cr	metroprt12	53%
HIGH 54.3	ROSHAN LAL	000006117161	39	₹2.45 Cr	₹0.37 Cr	INDUS01	74%
HIGH 57.6	ATMA SINGH	000004735685	38	₹2.39 Cr	₹0.07 Cr	capitol1	90%
HIGH 52.1	BHAJAN LAL	000006965080	39	₹2.38 Cr	₹0.00 Cr	capitol1	67%
HIGH 54.9	PITRAM SINGH	000005476164	38	₹2.37 Cr	₹0.02 Cr	sksoni	79%
HIGH 56.7	RAJINDER SINGH	000005778375	40	₹2.35 Cr	₹0.26 Cr	msshshb1	85%
HIGH 53.1	HARGOPAL	000004267450	37	₹2.32 Cr	₹0.20 Cr	guruhari	73%
HIGH 48.5	RAMESH CHAND	000004086189	40	₹2.31 Cr	₹0.00 Cr	NULL	52%
HIGH 52.4	KRISHAN PAL YADAV	000004356844	32	₹2.29 Cr	₹0.01 Cr	jaypeerc2	75%
HIGH 48.8	AMARJIT SINGH	000003397402	37	₹2.25 Cr	₹0.01 Cr	guruhari	57%

Single-hospital lock-in

Natural medical histories spread across providers. Near-perfect single-hospital concentration over dozens of high-value admissions is the strongest repeat-claimant signal — it points to a fixed beneficiary-hospital relationship that should be examined for any financial arrangement. Examples: MALKIT SINGH (59 claims, 85% at FHLDH123); KASHMIR SINGH (52 claims, 83% at echsfeha); RAJINDER SINGH (40 claims, 85% at msshshb1).

Q13F · SYSTEM-LEVEL

Extreme Bulk Claim Injection

Hundreds of intimation IDs for one card on one day. Tiers: >300 = system compromise, 100–300 = account abuse, 50–100 = watch

Table Q13f.1 — Bulk-injection events (same-day mass intimation)

Beneficiary	Card	Hospital	Date	Intim.	ID Range	Tier
GURJIT SINGH	CM000002580629	p.fatesahi	2022-08-04	546	21939246–21939907	SYSTEM COMPRO MISE
P SHYAM SUNDER	MU000002699039	pol.1928	2024-09-14	476	32661117–32661631	SYSTEM COMPRO MISE
NARENDRA SINGH	DL1000007590557	p.lodhird1	2024-12-12	349	34229861–34230245	SYSTEM COMPRO MISE
RAPURUVENKATASUB BAI...	HY000000136855	p.nello1	2024-08-10	330	32051504–32052554	SYSTEM COMPRO MISE
SHREE RAM GODIYALA	KL000003494970	nbnc2012	2026-02-26	315	43103293–43103870	SYSTEM COMPRO MISE
JALESHWAR RAI	000009122940	p.chapra	2025-07-19	257	38480768–38483104	ACCOUN T ABUSE
GURDIAL SINGH	CM000006679942	hos.4458	2025-07-18	176	38457875–38458066	ACCOUN T ABUSE
SURENDER KUMAR DHIN...	DL1000005286568	p.shakurb	2025-08-04	131	38796043–38797429	ACCOUN T ABUSE
THOMAS MATHEW	TR000005523638	muthmed	2026-03-12	108	43403614–43403782	ACCOUN T ABUSE
KURIAN V M	TR000002099834	muthmed	2026-03-07	84	43283816–43283906	WATCH
RAVINDRA NATH PAL	LU000001767189	9453698544	2024-07-27	82	31817295–31826023	WATCH
JAGRUP SINGH	JM000004151522	p.palam	2021-10-27	75	19043261–19043343	WATCH
RAMBAHADUR SINGH	DL1000006342019	SMH@D1	2024-09-07	74	32544983–32545161	WATCH
MOHANAN P	KO000003204217	p.thrissur	2026-03-17	72	43514670–43514832	WATCH
THOMAS J	TR000003125639	p.mavelik	2024-05-09	71	30491190–30491262	WATCH
DOST MOHAMMED	000009227651	p.chapra	2025-07-18	68	38460060–38460216	WATCH

Why this matters

The top event — **GURJIT SINGH** at **p.fatesahi** on 2022-08-04 — created **546 intimation IDs in a single day**, spanning IDs 21939246–21939907 (near-consecutive). Creating hundreds of records in one day is impossible through manual web-portal entry; near-consecutive ID ranges are the signature of programmatic batch insertion — portal credential compromise or insider database access, not ordinary claim fraud.

These 19 events span **9 distinct card-prefix codes** (each encoding an issuing ECHS region, including Delhi, Mumbai, Hyderabad, Kerala) — so this is not a single compromised login but a multi-region pattern. Response: pull portal access logs (IP, session, timestamps) for these dates and escalate to ECHS IT / CERT-In.

Conclusions

Across the last five years, **74,593 claims above ₹5 lakh** represent **₹5,630.76 Cr** of gross exposure, of which **₹1,785.08 Cr (31.7%)** was removed at audit — a deduction rate far above routine norms and evidence that high-value billing is systematically inflated.

The single largest issue is **data integrity**: 23,959 high-value claims (₹1,729.34 Cr) carry no traceable hospital ID. Fraud then concentrates in three places — **providers** (a handful of logins and the ECHS-polyclinic channel drive most of the absolute deduction), **beneficiaries** (chronic claimants with extreme admission frequency and near-total single-hospital lock-in), and a **system-security** exposure (same-day bulk injection of hundreds of intimation IDs across multiple regions, indicating credential compromise rather than ordinary fraud).

The composite risk score turns these signals into a single ranked queue, so auditors can act on the CRITICAL/HIGH tier first. Because every score is a transparent weighted sum, each ranking can be explained and defended. None of these findings is a conviction — they are prioritised leads, and each must be confirmed against physical records (discharge summaries, portal access logs, service records) before recovery or enforcement.

Consolidated Risk Register & Recommendations

Table R.1 — Risk register

Fraud signal	Scale	Exposure	Risk
High-value claims (>₹5L)	74,593 claims	₹5,630.76 Cr	CRITICAL
100% deduction claims	9,717 claims	billed beyond ceiling	CRITICAL
Hospital overcharging (>25% ded)	392 logins	see scorecard	CRITICAL
ECHS polyclinic abuse	4,499 claims	₹237.10 Cr deducted	HIGH
Chronic repeat claimants	3,135 cards	≥3 high-value claims	HIGH
Same-day duplicates	71 groups	double billing	CRITICAL
Bulk claim injection	19 events	5 system-compromise	CRITICAL
Ghost / anomalous hospital IDs	23,959 claims	₹1,729.34 Cr	HIGH

Table R.2 — Strategic recommendations (priority · SLA · named case)

Priority	SLA	Recommendation	Action
CRITICAL	Immediate	Portal-security investigation of bulk injection	GURJIT SINGH created 546 intimation IDs in one day at p.fatesahi. Pull portal access logs for all 19 events, suspend the logins, escalate to ECHS IT / CERT-In.
CRITICAL	7 days	Recover same-day duplicate payments	BHISHAM DEO SHUKLA: 2 identical ₹32.73 L claims on 2022-11-11 at 8983336860. Confirm which were paid, recover the surplus, sweep the login.
CRITICAL	14 days	Physically verify the top chronic claimant	BHUPAL SINGH — 117 high-value claims at fortis@D1. Obtain discharge summaries for a 20-claim sample; reconcile admission dates / ICU notes.
CRITICAL	Immediate	Investigate possible card-ID manipulation	AMRIT LAL CHADHA on two near-identical cards (DL1000004517957 / DL2000004517957). Verify against the service record — one pensioner or two?
HIGH	60 days	Empanelment & billing audit of top-deduction login	parkhosg — ₹64.43 Cr deducted across 2,399 claims (224 at 100%). Audit package rates and a 10-claim line-item sample.
HIGH	60 days	Segregate ECHS polyclinic claim channels	Polyclinic logins carry 4,499 high-value claims. Restrict them to OPD/pharmacy; quarantine high-value IPD claims submitted through them.
HIGH	30 days	Reject invalid hospital identifiers at intake	23,959 claims (₹1,729.34 Cr) carry NULL/phone-like IDs. Block submission where the hospital ID is not a valid portal login.
MEDIUM	Ongoing	Operationalise the risk score	Run the composite score nightly and route CRITICAL-band claims to auditors before settlement.

This report is produced by an automated screening system. Every flagged case is an investigative lead, not a confirmed finding, and must be reviewed by a qualified auditor before any action is taken.

Prepared by IIT Kanpur — Data Analytics & Fraud Intelligence Division | 05 June 2026 | RESTRICTED